

MediGuard Health Plan — Medical Policy Bulletin

Clinical Policy Department | Bulletin #MPB-2024-Q1 | Effective January 2024

This bulletin provides updated medical policies and clinical criteria for select procedures. These policies supplement the Prior Authorization Guidelines and apply to all MediGuard plan products. Providers should reference this bulletin for the most current medical necessity criteria.

Policy: Total Joint Replacement — Medical Necessity Criteria

Policy ID: MPC-2024-ORTHO-001

Applicable Codes: CPT 27447 (Total Knee Arthroplasty)

Total knee replacement is considered medically necessary when ALL of the following criteria are met: (1) Radiographic evidence of joint space narrowing, osteophytes, or bone-on-bone articulation consistent with Kellgren-Lawrence Grade III or IV; (2) Functional limitation documented by a validated outcome score (WOMAC, KOOS, or equivalent) indicating moderate to severe impairment; (3) Failure of conservative management for a minimum of 6 months including physical therapy, pharmacological treatment, and at least one intra-articular injection; (4) Patient BMI below 45 kg/m². Coverage: 70% of allowed amount after deductible. Estimated patient responsibility for in-network facility: \$8,500. Authorization valid 90 days.

Policy: Advanced Neuroimaging — Appropriate Use Criteria

Policy ID: MPC-2024-NEURO-003

Applicable Codes: CPT 70553 (MRI Brain with and without Contrast)

Brain MRI with and without contrast is considered medically necessary for: (a) New onset seizures in adults; (b) Suspected intracranial neoplasm based on clinical presentation or abnormal CT findings; (c) Monitoring of known intracranial pathology at intervals recommended by treating specialist; (d) Evaluation of demyelinating disease (e.g., multiple sclerosis); (e) Pre-operative neurosurgical planning. NOT medically necessary for: routine evaluation of chronic headache without red flags, screening in asymptomatic patients, or repeat imaging without interval change in symptoms. Coverage: 85% of allowed amount. Estimated patient out-of-pocket: \$320. Prior authorization required with clinical justification.

Policy: Knee Arthroscopy — Surgical Indications

Policy ID: MPC-2024-ORTHO-005

Applicable Codes: CPT 29881 (Arthroscopy, Knee, Surgical; with Meniscectomy)

Knee arthroscopy with meniscectomy is considered medically necessary when: (1) MRI confirms meniscal tear with clinical correlation; (2) Patient reports mechanical symptoms (locking, catching, giving way) for at least 6 weeks; (3) Conservative management has failed (physical therapy, NSAIDs, activity modification). Special consideration for patients over 50: when significant osteoarthritis is present on imaging, the provider must document that mechanical symptoms are primarily attributable to the meniscal pathology rather than the degenerative joint disease. Coverage: 80% of allowed amount after deductible. Estimated patient responsibility: \$2,200. Prior authorization required. MRI report must be submitted with the authorization request.

Policy: Outpatient Psychotherapy — Benefit Parameters

Policy ID: MPC-2024-BH-002

Applicable Codes: CPT 90837 (Psychotherapy, 60 Minutes)

Outpatient psychotherapy is covered under the mental health parity provision. 60-minute individual psychotherapy sessions (CPT 90837) are covered at 75% of the allowed amount with a \$25 copay per session. Annual session limit: 30 sessions per calendar year. Sessions beyond the annual limit require medical necessity review and documentation of treatment plan with measurable goals. No prior authorization required for sessions within the annual limit. Eligible provider types: licensed psychiatrists, psychologists, licensed clinical social workers (LCSW), and licensed professional counselors (LPC). Telehealth delivery is covered at the same rate as in-person visits. Estimated patient out-of-pocket per session: \$50.

Policy: Upper Gastrointestinal Endoscopy — Coverage Criteria

Policy ID: MPC-2024-GI-001

Applicable Codes: CPT 43239 (Upper GI Endoscopy with Biopsy)

Upper GI endoscopy with biopsy is covered at 90% of allowed amount when performed for diagnostic evaluation of: persistent dysphagia, refractory GERD (failed 8-week PPI trial), suspected Barrett's esophagus, upper GI bleeding, unexplained iron deficiency anemia, or surveillance of known Barrett's esophagus per ACG guidelines. No prior authorization required when clinical indication is documented. Estimated patient responsibility: \$175. Facility-based procedure; non-facility setting not applicable. Repeat endoscopy within 12 months requires documentation of new or worsening symptoms.